

MARY'S HANDS NETWORK

Volunteer Doula Field Guide

Communication Guide

2026 Edition | Authorized Internal Use Only

A Note Before You Begin

Everything in this guide comes back to one idea: your job is to help your client find her voice, not to be her voice for her.

Communication is not just about being friendly or staying in touch. In this work, how you say something, when you say it, who you say it to, and what you choose not to say are all clinical decisions. They shape whether your client feels supported or abandoned. They determine whether the nursing staff trusts you in the room. They are the difference between a doula who empowers and one who accidentally takes over.

This guide gives you the language, the templates, and the real-world scenarios you need to communicate well at every stage of the client relationship. Read it before your first client. Come back to it when you are not sure what to say. The situations in here are not hypothetical -- they happen at births in our network regularly.

You are not expected to handle everything perfectly. You are expected to know when to speak, when to stay quiet, and when to contact your volunteer coordinator.

Part 1: Communication Standards

As an MHN volunteer doula, you communicate in three directions: with your teammate, with your client, and with MHN staff. Each direction has its own purpose, its own tools, and its own time expectations. Knowing the difference keeps things from falling through the cracks.

Direction 1: Doula to Doula

Purpose: Keep your team aligned so your client never feels the gap between you.

Your client was matched with a two-person team on purpose. She should never feel like she has one doula; she should feel like she has a team that moves together. That only happens if you and your teammate are communicating consistently, honestly, and early.

STANDARDS

- Respond within 24 hours for routine updates. Respond immediately for anything time-sensitive; coverage requests, labor calls, or concerns about a client.
- Never let a coverage gap go unaddressed. If you cannot make a visit or cover a shift, say so right away, not the night before, not the morning of.
- If you and your teammate disagree about something related to a client, address it directly and calmly between yourselves first. If it is not resolved, contact your volunteer coordinator. Do not let disagreements affect your client's care.
- Assume good intent. Your teammate is doing her best, just like you are.

Direction 2: Doula to Client

Purpose: Build trust, maintain connection, and make sure she never feels alone.

Your client chose MHN because she wanted support. Every message you send her is a reminder that she made the right call. Every missed check-in is a small erosion of that trust and for clients with complicated home situations or limited social support, that erosion can be significant.

STANDARDS

- Reach out first. Do not wait for her to contact you between visits. The Field Guide says every two weeks minimum, that is the floor, not the goal.
- Set your availability clearly at your first virtual visit. Let her know your general hours and what to do if something urgent comes up outside of those hours.
- Never leave a message unanswered without acknowledging it. Even if you do not have an answer yet, a quick response lets her know she was heard.
- Routine questions and check-ins: respond within 24 to 48 hours.
- Anything that sounds urgent: respond within a few hours.
- A labor call or active labor message: respond immediately.

Direction 3: Doula to MHN Staff

Purpose: Keep operations running and make sure the right people know what they need to know.

Your volunteer coordinator is your first point of contact for everything, scheduling questions, client concerns, documentation issues, teammate conflicts, and any situation where you are not sure what to do. That is their job. You are not bothering them. You are doing yours.

STANDARDS

- Routine requests and updates: respond within 24 to 72 hours.
- Urgent matters: same day, as soon as possible.
- Use email for official communication that needs a record, confirming a match, or reporting a concern in detail.
- Use text for anything urgent during a birth or active situation, or when you need quick guidance.
- The one rule that covers everything: if something does not feel right, contact your volunteer coordinator before you decide it is not a big deal. It is much easier to say never mind than to circle back after something went wrong.

Communication Tools at a Glance

Situation	Best Tool	Response Time
Routine doula-to-doula update	Text or group chat	Within 24 hours
Coverage request	Text or group chat	Immediately
Client check-in	Text	Every 2 weeks minimum
Client question (non-urgent)	Text	Within 24 to 48 hours
Client in labor	Text or call	Immediately
Reporting a concern to staff	Text (urgent) or email	Same day or within 24 hours
Official match confirmation	Email	Within 48 hours

Part 2: Templates You Can Use Right Now

These are ready to copy, paste, and personalize. Every one of them is written the way MHN expects doulas to communicate: warm, clear, and focused on the client.

First contact after a match

Hi [Name]! My name is [Your Name] and I am one of your doulas through Mary's Hands Network. I am so glad to be matched with you. I would love to set up a time to connect -- even just a quick call or video chat to introduce ourselves and get some time on the calendar. Does [day] around [time] work for you, or is there a time that is generally easier?

Follow-up if she does not respond (after 3 to 4 days)

Hi [Name], just wanted to follow up on my message from [day]. No pressure at all -- I know pregnancy is busy and life happens. Whenever you are ready to connect, I am here. You can reach me at this number anytime.

Biweekly check-in

Hey [Name], just thinking about you this week. How are you feeling? How is baby moving? No need to respond right away -- just wanted you to know we are here.

Following up on something from last visit

Hi [Name]! Just checking in -- did you ever get a chance to [connect with the lactation consultant / call WIC / schedule that appointment]? Let me know if you want help with any of that.

Visit confirmation (the day before)

Hi [Name], just confirming we are still on for [day] at [time] at [location]. Looking forward to seeing you. Text me if anything comes up.

After a visit

It was so good to see you today. Thank you for your time and for sharing so much with us. Our next visit is scheduled for [date]. In the meantime, do not hesitate to reach out -- we are here.

Coverage request to your teammate

Hey, I wanted to give you a heads up that I am not going to be available on [date / time]. Is there any chance you can cover? If not, let me know right away so we can loop in our volunteer coordinator early. I want to make sure [client's first name] has coverage.

Sharing a client update with your teammate

Hi -- quick update on [client's first name]: [brief, relevant, non-sensitive update]. Nothing urgent, just wanted to make sure you were in the loop. Let me know if you have any questions or want to talk through next steps.

Reporting a concern to your volunteer coordinator

Hi, I wanted to bring something to your attention from [today's visit / recent communication] with my client. [Brief, factual description of what happened or was shared.] I did not make any commitments or next steps yet -- I wanted to check in first. What would you recommend?

Declining a match

Thank you for thinking of me for this match. I am not available at this time due to [brief reason]. I appreciate you checking in and I look forward to being available for the next one.

Part 3: Prenatal Scenarios

Each scenario below includes what is happening, what not to do, what to say or do instead, and what to do after. These are not hypothetical -- they happen regularly in MHN's network.

Scenario 1	She Does Not Respond to Your First Message	Prenatal
You were matched five days ago. You sent a warm first message and have not heard back.		
Do not: • Send multiple messages in quick succession. • Assume she dropped out. • Ask your volunteer coordinator to rematch you without trying again.		
Wait two to three days after your first message, then try again using a different format. If you texted, try a call. Keep your second reach-out light and low-pressure.		
What to say: <i>"Hi [Name], this is [Your Name], your doula from Mary's Hands Network. I just wanted to try giving you a call to introduce myself. No worries if now is not a good time -- you can call or text me back whenever works for you. I am looking forward to connecting."</i>		
After one week of no response: Contact your volunteer coordinator. Do not continue reaching out indefinitely without looping in staff. A client going quiet is often a sign that something difficult is happening in her life, not that she does not want support.		

Scenario 2	She Goes Quiet Between Visits	Prenatal
Your client was responsive and engaged, but for the past few weeks she has not replied to your check-ins. She missed a scheduled visit and has not rescheduled.		
Do not: • Assume she is fine and wait it out. • Assume she dropped out and stop reaching out. • Keep sending messages without alerting staff.		
Try one more time using a different format from what you have been using. Keep the message brief and low-pressure.		
What to say:		

"Hi [Name], just wanted to check in. No pressure at all -- I know things get busy. Whenever you are ready to connect, we are here. If something has come up and you need anything at all, please reach out or call the MHN Doula Hotline at (225) 424-7532."

If she still does not respond:

Contact your volunteer coordinator right away. Staff have additional ways to reach clients and can escalate if there is a safety concern. The moment a client feels abandoned by her doula team is often the moment she needs support the most.

Scenario 3

A Partner or Family Member Dominates Communication

Prenatal

At your virtual pre-birth visit, your client's partner answers most of the questions before she can respond. She looks a little checked out. Later, you start receiving texts from the partner about the birth plan -- not from the client herself.

Do not:

- Direct your answers to the partner because it is easier.
- Assume the client is fine with this arrangement.
- Ignore the pattern.

In the visit, gently redirect your attention back to your client by name. In text communication, respond to the client directly, not to the partner.

What to say in the visit:

"That is so helpful to know. And [Client's Name], what do you think about that? What feels most important to you?"

If the partner texts you:

"Thanks for reaching out. I want to make sure [Client's Name] is in the loop too -- would it be okay if I follow up with her directly so we can all stay on the same page?"

After the visit:

Note what you observed in your form. If the dynamic felt controlling rather than just engaged, mention it to your volunteer coordinator.

Scenario 4

She Asks You to Make a Decision for Her

Prenatal

At the first in-person visit, your client says: "You have done this a lot. Just tell me what I should do about my birth plan. I trust you more than my doctor."

Do not:

- Tell her what you would do.
- Share what most clients choose.
- Give her a recommendation, even a gentle one.

Acknowledge how much trust she is placing in you, and redirect that trust toward her own instincts.

What to say:

"That means so much to me, and I take that trust seriously. But this is your birth, and your instincts about your own body and your own baby matter more than anything I could tell you. What I can do is help you think through your options so you feel confident making the call yourself. What feels most important to you right now?"

If she pushes: "I hear you -- it can feel overwhelming. Let's take it one thing at a time. Tell me what you already know you want, and we will go from there."

Scenario 5 She Discloses an Unsafe Home Situation

Prenatal

During your first in-person visit, your client becomes quiet and tells you that her partner has been "getting really bad" since she got pregnant. She does not use the word abuse, but she says she is scared to go home some nights.

Do not:

- Try to assess the situation yourself.
- Ask detailed follow-up questions about the relationship.
- Make promises about what MHN can do.
- Minimize what she shared or pivot quickly to the next question.

Stay calm. Do not overreact. Thank her for trusting you. Stay present. After the visit, contact your volunteer coordinator the same day.

What to say in the moment:

"Thank you for telling me that. That took courage and I want you to know I hear you. You deserve to feel safe. I am not going to pretend I have all the answers right now, but I want to make sure you have support. Can I ask -- are you safe right now, today?"

If she says yes: "Okay. I am going to make sure our team knows so we can connect you with some additional resources. You do not have to navigate this alone."

After the visit:

Contact your volunteer coordinator immediately. Document what she shared factually and accurately in your visit form. This is not a reason to stop supporting her -- it is a reason she needs more support, not less.

Scenario 6 She Cannot Nail Down a Visit Time

Prenatal

You have been trying to schedule the first in-person visit for two weeks. She agrees to a time, then cancels. She suggests a time, then does not confirm. You have sent several messages.

Do not:

- Keep pushing without acknowledging the pattern.
- Get frustrated in your tone.
- Give up and wait for her to reach out.

Make it as easy as possible. Offer two or three specific options across different days and times rather than asking her to propose something.

What to say:

"Hey [Name] -- I know life is busy, especially right now. I want to make this as easy as possible. Would any of these work for you? [Option A], [Option B], or [Option C]? If none of those are right, just let me know what time of day generally works best for you and we will figure it out from there."

If scheduling remains a barrier:

Let your volunteer coordinator know so she can help facilitate. Some clients need staff involvement to get things moving -- that is not a failure, it is just how it goes sometimes.

Part 4: Labor and Birth Scenarios

The guiding principle for everything in this section:

Your client speaks for herself. Your job is to help her feel strong enough to do it.

Scenario 1

You Misjudge When to Come

Labor and Birth

Your client calls to say her contractions are 8 to 10 minutes apart. She sounds calm. You tell her to call you when they are closer together. Four hours later, she texts to say she already delivered. She and her partner are upset.

Early labor, especially during an induction, can be long and emotionally draining even when contractions are not intense or close together. A client who labored alone for six hours before you arrived did not have a doula -- she had a person who showed up for the birth.

Do not:

- Use contraction frequency as the only measure of when she needs you.
- Wait for her to tell you she is ready.

When she calls, ask open questions before making any assessment.

What to say:

"Tell me what is happening right now. How are you feeling? What does your body feel like between contractions, not just during them? I want to make sure we are there when you need us -- and I would rather come early than have you going through this alone."

If you cannot be there right away: Stay in close contact. Text between contractions. Let her know exactly when you are coming. Do not go silent.

Scenario 2

She Says "I Can't Do This Anymore"

Labor and Birth

Your client is in active labor, possibly approaching transition. She is exhausted and overwhelmed. She may or may not be asking for an epidural.

"I can't do this" is often a sign of transition -- the most intense and shortest phase of labor. It is also a completely valid expression of how hard labor is, and it deserves to be met with presence, not a pep talk.

Do not:

- Tell her she is almost done.
- Tell her she can do it without an epidural.
- Tell her what you think she should do.

What to say:

"I hear you. This is really hard. You are doing it -- right now, with every breath. Let's just get through this one."

If she asks about an epidural: "That is absolutely your call to make. Would you like me to let the nurse know you want to discuss it, or do you want a few minutes first?"

Never say:

"You are so close!"

"You said you did not want one."

"Just try one more contraction."

"Think about your birth plan."

None of these are your call. Her body is her decision.

Scenario 3**Labor Slows or Stalls***Labor and Birth*

Your client has been at the same dilation for several hours. The energy in the room is tense. The nursing staff is beginning to talk about interventions. Your client looks defeated.

Do not:

- Tell her what the nurse said was wrong.
- Give your opinion on the interventions being discussed.
- Tell her what you would do.
- Try to speed things up yourself.

Focus entirely on her experience in this moment. Keep her comfortable, keep her moving if she can, and help her feel less alone in the waiting.

What to say:

"Labor is not always a straight line. Your body is working, even when it does not feel like it. What does your body want to do right now?"

If interventions are offered and she looks uncertain: "Would it help to have a minute to think about it, or do you feel ready to decide? There is no wrong answer."

If she asks what you think she should do: "This is your call and I am going to support whatever you decide. What matters most to you right now?"

What to do physically: try a position change, encourage her to empty her bladder, offer counter pressure, walking, or the birth ball if appropriate. Keep the room as calm as possible.

Scenario 4**She Changes Her Mind About the Epidural***Labor and Birth*

Your client planned an unmedicated birth. You talked through it at length during your visits. She is now mid-labor and asking for an epidural.

Do not:

- Remind her of her birth plan.
- Suggest she try one more contraction first.
- Express any hesitation or disappointment, even subtly.
- Say "Are you sure?"

Support her decision immediately, fully, and without reservation. This is not a setback. This is her exercising her right to make decisions about her own body in real time.

What to say:

"Absolutely. I'll let the nurse know you are ready to discuss the epidural. I am right here."

Then tell the nurse. That is it.

After she has the epidural: your support does not stop. A client with an epidural still needs her doula. Help with positioning, keep her company, and keep supporting her emotionally through to delivery.

Scenario 5 The Birth Plan Changes Mid-Labor

Labor and Birth

The provider comes in and recommends a change from the birth plan -- Pitocin to augment labor, breaking the water, or another intervention. Your client looks at you instead of the provider.

Do not:

- Speak to the provider on her behalf.
- Say anything that sounds like a recommendation for or against the intervention.
- Reference what she said she wanted in her birth plan without her asking you to.

Help her direct her attention back to her provider and her own voice.

What to say quietly to her:

"This is your decision. Do you want a minute to think, or do you have questions you want to ask first?"

If she says she has questions: "What do you want to ask them?" Let her form the question. Stay close.

If she asks for the BRAIN tool: walk through it with her quietly -- Benefits, Risks, Alternatives, what her Intuition says, and whether she needs more time.

If she asks what you think: "I am not able to tell you what to choose, but I can help you think it through. What is your gut telling you?"

Scenario 6 A Medical Emergency Fills the Room

Labor and Birth

Something changes suddenly -- fetal heart tones drop, there is unexpected bleeding, or the situation shifts in a way that brings multiple staff into the room quickly.

Do not:

- Stay where you are and continue comfort measures.
- Ask what is happening.
- Speak to the clinical staff.

Step back immediately. Get out of the way of the clinical team -- completely and without hesitation. This is not the moment for the BRAIN tool, for positioning, or for comfort measures. The medical team needs full access to your client.

Where to be and what to do:

Stay visible to your client from wherever you can safely stand. Make eye contact if you can. Be calm. Your presence is the support in this moment.

Say nothing to the clinical team. If your client reaches for you, hold her hand if you can do so without being in the way.

After the emergency: stay close. Let her talk if she wants to. Do not try to explain what happened -- that is the clinical team's job. Your job is to make sure she does not feel alone in the aftermath.

Scenario 7

A Partner or Family Member Becomes Verbally Abusive

Labor and Birth

You are at the birth. The client's partner is under enormous stress and begins directing that stress at you -- raising their voice, making dismissive or aggressive comments, or becoming hostile.

Do not:

- Absorb it and keep going.
- Confront them or escalate.
- Leave the building.

Step out of the room calmly. Do not escalate. Contact your volunteer coordinator immediately from the hallway.

What to say as you leave the room:

"I need to step out for a moment. I will be right back."

That is enough. You do not owe anyone an explanation in that moment.

Do not leave the building without notifying a staff member. You are still responsible for your client even if you cannot be in the room.

After you have spoken to your coordinator:

Follow their guidance. Your safety matters. You do not have to return to a situation that is unsafe for you. Your coordinator will help navigate what happens next.

Scenario 8

Neither Teammate Can Make It to the Birth

Labor and Birth

Despite your planning and your on-call schedule, neither you nor your teammate is available when your client goes into labor. She has delivered, or is in active labor, without doula support.

Do not:

- Send a long apology text.
- Wait until she is home from the hospital to reach out.

Get to the hospital as soon as possible. When you arrive, do not lead with an apology -- lead with presence.

What to say when you walk in:

"I am here. Tell me how you are."

Let her talk. Do not explain yourself unless she asks. Your job right now is to make her feel cared for and seen -- not to manage your own feelings about missing the birth.

Missing the birth is not the end of the relationship. What you do afterward is what she will remember.

Part 5: Postpartum Scenarios

Scenario 1	She Is Upset About Something That Happened at the Birth	Postpartum
During the 72-hour check-in, your client is emotional and frustrated. She felt like her preferences were ignored. She is not sure if she is angry at the medical team, at you, or at the whole situation.		
Do not: • Defend yourself. • Defend the medical team. • Tell her everything worked out because the baby is healthy. • Try to find the silver lining. • Tell her she should have spoken up more in the moment.		
Listen. Let her tell the whole story without interruption. Do not rush toward resolution.		
What to say: <i>"Tell me what happened. I want to understand what it was like for you."</i> <i>After she has talked: "That sounds really hard. I am glad you are telling me this."</i> <i>If she expresses frustration at you specifically: "Thank you for telling me. I hear you. Can you tell me more about what you needed from me in that moment?"</i> <i>Do not get defensive. Do not explain your reasoning. This conversation is not about you.</i>		

Scenario 2	She Seems to Be in Crisis	Postpartum
During the 72-hour check-in or the first postpartum visit, your client is not okay. She may be crying without being able to stop. She may be saying things that worry you -- that she cannot do this, that her baby deserves better, that she feels nothing. She may be expressing thoughts that feel like more than the typical postpartum adjustment.		
Do not: • Complete the EPDS and move on. • Tell her this is normal and it will pass. • Leave the visit without addressing what you are seeing. • Try to assess the severity of the situation yourself.		
Stay. Do not leave her alone if she is expressing thoughts of harm toward herself or her baby. Contact your volunteer coordinator immediately. If you believe there is an immediate safety risk, help connect her to emergency services.		

What to say:

"I hear you. What you are feeling is real and it matters. I do not want you to go through this alone. Can I help get some additional support for you today?"

If she resists: stay calm, stay present. Keep the line of communication open. Your job is not to fix the crisis -- it is to make sure she is not alone in it and that the right people are involved.

Scenario 3

She Is Struggling But Not Asking for Help

Postpartum

Your client is clearly exhausted and overwhelmed at the first in-person postpartum visit. But when you ask how she is doing, she says "fine" or "we are managing." The house is chaotic. She looks like she has not slept in days. She keeps apologizing for the mess.

Do not:

- Take "fine" at face value and move through your questionnaire.
- Wait for her to ask for help.

Offer something specific and concrete without waiting for permission.

What to say:

"Why don't you go lie down for a little while -- I will stay with the baby while you rest."

"Can I do these dishes while we talk? It would make me feel useful."

"What would help you most right now -- is it sleep, or food, or just having someone here?"

Practical, present, hands-on support is one of the most powerful things you offer in the postpartum period. Do not wait to be invited. Show up and do something real.

Scenario 4

She Goes Silent After Birth

Postpartum

Your client was engaged and communicative during pregnancy, but since the birth she has not responded to your messages. She missed the scheduled 72-hour check-in call.

Do not:

- Assume she is fine and busy with a new baby.
- Stop reaching out.
- Wait until her first in-person visit.

Try a different format -- a call instead of a text, or a voice note. Keep the message brief and warm.

What to say:

"Hi [Name], just thinking about you and wanted to check in. No pressure at all -- whenever you are ready to connect, we are here. If you need anything before then, please reach out or call the MHN Doula Hotline at (225) 424-7532."

If there is still no response after two attempts:

Contact your volunteer coordinator immediately. The postpartum period is one of the most vulnerable times in a new mother's life. A client going quiet after birth is a signal that needs attention -- not an assumption that everything is okay.

Part 6: When to Escalate

Use this as a quick reference when you are not sure what to do next.

Handle it yourself	Contact your volunteer coordinator	Call 911 first
Scheduling challenges with your client	Client who has not responded to two consecutive check-ins	Client expressing immediate intent to harm herself or her baby
A routine check-in that goes unanswered once	Client who disclosed a safety concern, housing instability, or mental health struggle	Client in immediate physical danger
A client who is emotional but not in crisis	A conflict with your teammate that you cannot resolve on your own	Medical emergency where clinical staff are not present
Disagreement with your teammate that you have not yet tried to address	A birth situation that felt complicated, uncomfortable, or unclear	
Uncertainty about which comfort measure to try next	A coverage gap you cannot fill	
A client who changes her mind about her birth plan	Any situation where you are unsure whether what you said or did was within your scope	
	A partner or family member whose behavior concerned you	
	Verbal abuse toward you at a birth	
	Any time you left a birth early or under difficult circumstances	

One Last Thing

You are going to face situations that are not in this guide. Birth is unpredictable. People are complicated. Something will happen that you were not prepared for, and in that moment you will have to make a call.

When that happens, come back to the two things that never change:

Your client speaks for herself. You help her find the strength to do it.

And when you are not sure what to do -- contact your volunteer coordinator.